SACTCHECK · NCCP 00209

# My complete treatment handbook

## Modified FOLFOX-6

All information can be read on paper. Keep this with your treatment record.

> Use the urgent instructions and contact numbers in the following companion pages. Example telephone numbers must be replaced by your team before patient use. Your individual treatment plan takes precedence.

## Contents


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SACTCheck v0.84.33 · Clinical, pharmacy and patient review pending. Formal consent is supplied separately.

# Modified FOLFOX-6

Modified FOLFOX-6 combines oxaliplatin, folinic acid and fluorouracil. It can be used after colon-cancer surgery or to treat advanced colorectal, stomach or gastro-oesophageal-junction cancer. The aim and duration depend on the indication; ask which applies to you.

**Gastrointestinal cancers** · NCCP 00209 v10a · Document revised 4 November 2026

**Your personal plan:** this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

## My overall treatment

Day 1: oxaliplatin, folinic acid and fluorouracil, followed by a 46-hour fluorouracil pump. A cycle lasts 14 days.

**14-day treatment cycle**
Day 1: oxaliplatin, folinic acid and fluorouracil, followed by a 46-hour fluorouracil pump. A cycle lasts 14 days.

**Review**
Course length, response scans and next steps follow your individual plan.

## A typical 14-day cycle

| Day 1 | Day 2–3 | Day 4–7 | Day 8–14 |
|---|---|---|---|
| Infusion and pump connection | The pump is delivering treatment | After disconnection | Between visits and the next review |

Timing may change after review. Side effects can occur on any day; there are no guaranteed safe days.

## My tests and checks

| When | What is checked | Timing / reason |
|---|---|---|
| **Before** | **DPD and baseline bloods**<br>Required by regimen | Confirm DPD testing, blood count, kidney and liver results before starting. |
| **During** | **Blood count / kidneys / liver**<br>Required by regimen | Before each cycle. Review symptoms and medicines. |
| **After / review** | **Recovery, scans / markers**<br>Team-agreed plan | Follow the individual cancer plan; this regimen does not prescribe a universal scan or marker interval. |

## Between treatments

- Report fever, sudden illness, diarrhoea, mouth pain or poor intake promptly.
- Symptoms can occur during treatment, between visits and after completion.

Confirm your prescription, supportive medicines, tests and working contact numbers.

**When to get help**

**Call 112 / 999** for severe breathing difficulty, severe chest pain, collapse or a seizure.

**Call oncology immediately:** temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321
**EXAMPLE ONLY — team must replace both numbers before patient use.**


Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

# What to watch for

You do not need to know which medicine caused a symptom before calling.

### 1 Mouth / eating

**Fluorouracil**

Report painful ulcers or difficulty eating and drinking.

### 2 Chest / heart

**Fluorouracil**

New chest pain needs urgent assessment, including during an infusion.

### 3 Bowel / hydration

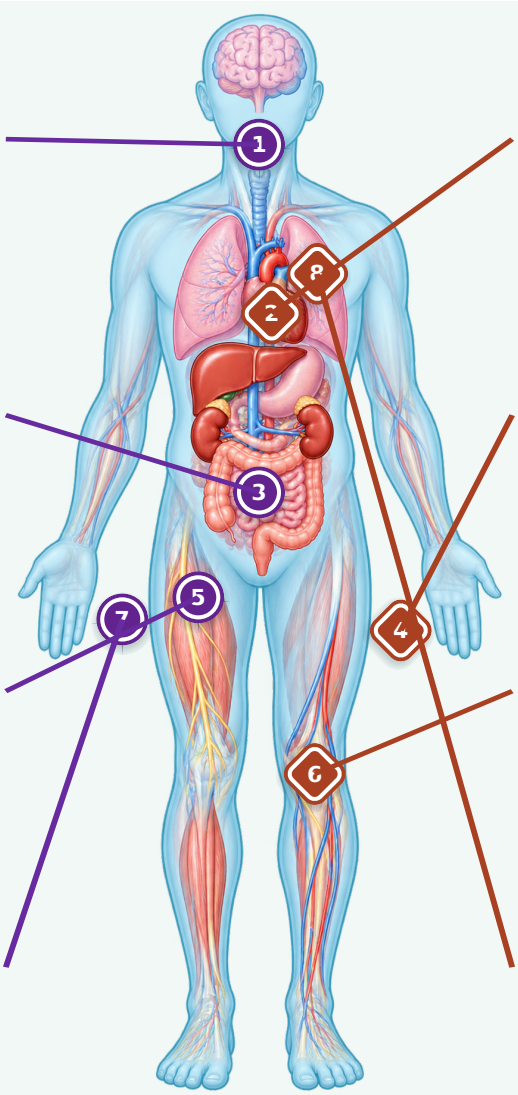
**Fluorouracil**

Report diarrhoea, increased stoma output, severe pain, blood or dehydration.

### 4 Hands / feet

**Fluorouracil**

Report painful redness, cracking or peeling that affects daily activity.




### 5 Blood / infection

**Chemotherapy**

Fever, shivers or sudden illness need immediate oncology advice.

### 6 Energy / daily life

**Treatment**

Report fatigue that limits usual activity; ask for practical and nutrition support.

### 7 Nerves

**Oxaliplatin**

Cold can trigger tingling. Report lasting numbness or difficulty with buttons or walking.

### 8 Throat / breathing

**Oxaliplatin**

Report throat tightness or breathing difficulty promptly. Tell staff immediately about infusion-site pain or swelling.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

New chest pain, breathing difficulty, severe diarrhoea, inability to drink, reduced urine, uncontrolled bleeding or a rapidly worsening symptom needs prompt assessment.

**When to get help**

**Call 112 / 999** for severe breathing difficulty, severe chest pain, collapse or a seizure.

**Call oncology immediately:** temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**

# Keep your treatment details to hand

**Gastrointestinal cancers** · NCCP 00209 v10a · Document revised 4 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

**FRONT / CONTACTS**

## My Treatment Card

SACTCheck

Name: _____

**Modified FOLFOX-6**

Day 1: oxaliplatin, folinic acid and fluorouracil, followed by a 46-hour fluorouracil pump. A cycle lasts 14 days.

Phase: _____ Cycle plan: team to confirm

**Daytime (EXAMPLE)** 087 12345678

**Out of hours (EXAMPLE)** 087 87654321

**Unwell or fever? Call oncology now.**
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

**BACK / TREATMENT RECORD**

## My cycle record

Rolling record — follow your agreed treatment plan.

| Cycle | Date / initials |
|---|---|
| | |
| | |
| | |
| | |

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

CUT outer border

**FOLD centre — do not cut**

## Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

## Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

### iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

### Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

## Your local contacts and resources

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**

Hospital / team: _____
Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

NCCP 00209 · VERSION 10A

# Your treatment, explained

**Gastrointestinal cancers** · NCCP 00209 v10a · Document revised 4 November 2026

**Your personal plan:** this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Modified FOLFOX-6

14-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

## When to get help

### Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

### Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. New chest pain, breathing difficulty, severe diarrhoea, inability to drink, reduced urine, uncontrolled bleeding or a rapidly worsening symptom needs prompt assessment.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

**Open your CUH contact / treatment card**

Sources: nccp · hse

YOUR COMPLETE REFERENCE

## Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- Your treatment plan
- Your 14-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Your port and treatment pump
- Tablets, visits and practical planning
- Your indication and treatment aim
- Living with the 46-hour pump
- What the checks are for
- Recognising treatment effects
- Living with treatment
- Words you may hear
- Supportive medicines
- Understanding the evidence
- Questions to ask

Sources

UNDERSTAND YOUR TREATMENT

# Your medicines and what they do

Modified FOLFOX-6 combines oxaliplatin, folinic acid and fluorouracil. It can be used after colon-cancer surgery or to treat advanced colorectal, stomach or gastro-oesophageal-junction cancer. The aim and duration depend on the indication; ask which applies to you.

PLATINUM CHEMOTHERAPY

## Oxaliplatin

Eloxatin



**Links DNA strands**

Oxaliplatin makes links within DNA. These interfere with copying and repair, which can stop a cancer cell dividing. Healthy tissues can also be affected.

Platinum enters cell → DNA links form → Copying is disrupted

**What to watch for**

Cold can trigger tingling or throat discomfort. Tell the team about persistent numbness, difficulty with buttons or walking, or infusion symptoms. New breathing difficulty always needs assessment.

Sources: nccp · patient

CHEMOTHERAPY MODULATOR

## Folinic acid

Calcium leucovorin



**Strengthens fluorouracil action**

Folinic acid helps fluorouracil remain bound to an enzyme needed to make DNA. In this regimen it increases fluorouracil activity and can also increase its unwanted effects. It is not a vitamin supplement to start yourself.

Folinic acid → Stronger enzyme binding → Fluorouracil effect increases

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### What to watch for

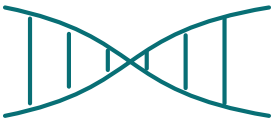
The combined treatment can increase mouth soreness and diarrhoea. Report these promptly. Rare allergic reactions can occur.

Sources: nccp · patient

CHEMOTHERAPY

### Fluorouracil

5-FU



**Disrupts cell building blocks**

Fluorouracil interferes with the production and use of DNA and RNA, the instructions cells need to grow. The infusion extends exposure over approximately two days.

Drug enters cell → DNA / RNA disrupted → Growth slows

**What to watch for**

Diarrhoea, mouth sores and skin soreness may become serious. Chest pain during the pump infusion needs emergency assessment. Unexpected severe early toxicity can occur even after DPD testing.

Sources: nccp · patient

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

**Explore your existing anatomy and side-effect map**

THE BIGGER TREATMENT PLAN

## Your treatment plan

INFUSION + HOME PUMP

### 14-day treatment cycle

D1 D2 D3 D14

Day 1: oxaliplatin, folinic acid and fluorouracil, followed by a 46-hour fluorouracil pump. A cycle lasts 14 days.

The treating team confirms the indication, schedule and duration. Changes are recorded on your own prescription.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

## Checks across the whole treatment course

The columns show the order of care, not equal lengths of time. Confirm who books each check, who reviews it and whom to call if it is missing.

| Check | Before starting | Each treatment cycle | Between doses | Decision / response review | Completion / change |
|---|---|---|---|---|---|
| **Blood count / kidneys / liver** | Baseline bloods; DPD before fluoropyrimidine | Repeat before each cycle | Earlier tests if unwell, dehydrated or toxicity develops | Results and recovery guide any delay or dose change | Follow unresolved abnormalities |
| **Symptoms and function** | Document existing symptoms and usual activity | Review worst symptoms since the last treatment | Use urgent contacts; do not wait for the next visit | Balance benefit, side effects and priorities | Agree recovery care and ongoing support |
| **Neuropathy / cold sensitivity** | Record existing nerve symptoms | Check persistence and effect on daily tasks | Report worsening numbness or cold-triggered throat symptoms | Consultant reviews the oxaliplatin component separately | Nerve symptoms can continue after treatment; arrange review |
| **Heart checks** | Review cardiac history; ECG when cardiac function is compromised or clinical findings warrant it. No automatic serial echocardiogram schedule is specified for this regimen. | Ask about new chest pain or breathlessness | Chest pain needs urgent assessment | Repeat heart tests if clinically indicated | Follow any heart problem; no fixed echo series |
| **Response / scans** | Confirm cancer setting, disease assessment and intended duration | Review clinical benefit and symptoms | New symptoms may bring review forward | Consultant-directed scans and response assessment | Agree follow-up, completion or another treatment |

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00209 v10a · official regimen

THE WHOLE TREATMENT JOURNEY

## Modified FOLFOX-6: treatment and checks

### Gastrointestinal cancers

Day 1: oxaliplatin, folinic acid and fluorouracil, followed by a 46-hour fluorouracil pump. A cycle lasts 14 days.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | Each treatment cycle | Between doses | Decision / response review | Completion / change |
|---|---|---|---|---|---|
| **Blood count / kidneys / liver** NCCP 00209 | Baseline bloods; DPD before fluoropyrimidine | Repeat before each cycle | Earlier tests if unwell, dehydrated or toxicity develops | Results and recovery guide any delay or dose change | Follow unresolved abnormalities |
| **Symptoms and function** NCCP 00209 | Document existing symptoms and usual activity | Review worst symptoms since the last treatment | Use urgent contacts; do not wait for the next visit | Balance benefit, side effects and priorities | Agree recovery care and ongoing support |
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| **Response / scans** NCCP 00209 | Confirm cancer setting, disease assessment and intended duration | Review clinical benefit and symptoms | New symptoms may bring review forward | Consultant-directed scans and response assessment | Agree follow-up, completion or another treatment |

## What each check is for

1

**Confirm your personal plan**

Before starting

Modified FOLFOX-6 combines oxaliplatin, folinic acid and fluorouracil. It can be used after colon-cancer surgery or to treat advanced colorectal, stomach or gastro-oesophageal-junction cancer. The aim and duration depend on the indication; ask which applies to you.

**Ask / act:** Agree medicines, treatment days, duration and monitoring.

Sources: NCCP 00209

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-04 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.32

## Day 1 → Day 14: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Day 1: oxaliplatin, folinic acid and fluorouracil, followed by a 46-hour fluorouracil pump. A cycle lasts 14 days.

| Day of cycle | 1 | 2 | 3 | 4 | 5 | 6 | 7 | 8 | 9 | 10 | 11 | 12 | 13 | 14 |
|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|
| **Day-ward medicines** | IV | | | | | | | | | | | | | |
| **Fluorouracil pump** | 46 hours; disconnect as arranged | | | | | | | | | | | | | |
| **Cold sensitivity** | Often early; may last longer | | | | | | | | | | | | | |
| **Blood counts / infection** | Timing varies; no guaranteed safe days | | | | | | | | | | | | | |

| Day of cycle | 1 | 2 | 3 | 4 | 5 | 6 | 7 | 8 | 9 | 10 | 11 | 12 | 13 | 14 |
|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|
| **Mouth / bowel / energy** | Record symptoms throughout | | | | | | | | | | | | | |

**Treatment and pump:** Day 1 treatment is followed by a 46-hour fluorouracil infusion. The team confirms the exact pump disconnection time.

**Blood counts:** Counts can fall between visits. The day of the cycle and how well you feel cannot rule out infection.

**Cold sensitivity:** Tingling or throat discomfort can occur early and may persist; follow the precautions your team gives you.

**Symptoms over time:** Mouth and bowel symptoms or fatigue can overlap. Numbness may accumulate across cycles; describe changes before the next dose.

> **Why these bands stay broad:** Bars show planned treatment and possible symptom patterns, not measured risk. Side effects can overlap and persist. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00209 v10a · official regimen ↗ · eviQ: patient information (supportive explanation; NCCP schedule takes priority) ↗

> **There are no guaranteed “safe days”.** Act on fever, sudden illness or a new serious symptom whenever it occurs. Do not wait for the next appointment.

**Read the whole cycle as text**

### Day 1: Infusion and pump connection

Symptoms and blood tests are reviewed. The day-ward medicines are followed by connection of a fluorouracil pump. Tell the nurse immediately about a reaction or pain at the infusion site. Before leaving, confirm the pump-care instructions, disconnection appointment and contact number.

### Day 2–3: The pump is delivering treatment

The infusion runs for about 46 hours from connection; the calendar day does not determine the exact finish time. Follow the instructions for keeping the line dry and secure. Avoid cold exposure if it triggers symptoms. Call for a leaking, disconnected or unexpectedly stopped pump. Do not reconnect, alter settings or disconnect it yourself unless specifically trained and instructed.

### Day 4–7: After disconnection

Nausea, diarrhoea, mouth symptoms, fatigue or cold sensitivity may continue. Use prescribed support. New symptoms deserve advice even if the pump has finished. Report poor intake, persistent diarrhoea or mouth pain early. Chest pain or major breathing difficulty requires emergency help.

### Day 8–14: Between visits and the next review

Blood counts can fall between visits. Numbness may accumulate over cycles. Describe any difficulty fastening buttons, walking or handling objects before more oxaliplatin. Bring the symptom passport and check the next appointment. Day 1 usually repeats 14 days after this cycle started, only if confirmed.

YOUR TREATMENT, STEP BY STEP

## Your port and treatment pump

From preparing the line to coming home after disconnection. Your team arranges the appointments and shows you what to do.



1 · Before treatment

**Choose and prepare the line**

The team explains whether a port or PICC suits your treatment. You receive instructions for the insertion appointment.



2 · Insertion and recovery

**A separate appointment may be needed**

For a port, numbing medicine is used and a small device is placed under the skin. Ask about sedation, transport and wound care.



3 · Day 1

**The nurse connects your pump**

After the day-ward medicines, the nurse checks and connects the fluorouracil pump. Ask for a demonstration before leaving.

4 · At home

### Treatment continues for about 46 hours

Carry the pump as shown. Keep the dressing dry and protect the line from pulling, heat and damage.

5 · As arranged

### Return for disconnection

A trained professional checks the infusion, disconnects the pump and cares for the line. The appointment is based on connection time.

6 · Between cycles

### The access device usually stays

A port needle can be removed while the port stays under the skin. A PICC remains in the arm with ongoing dressing and line care.

### Port or PICC: what is the difference?

A port is a small chamber beneath the skin, usually on the chest, attached to a tube ending in a large vein. A special needle goes through the skin into the port when it is used. A PICC enters a vein in the upper arm; a short section remains outside under a dressing. Both can deliver a home infusion. Your team helps choose the device and teaches its care.

### Before insertion and during recovery

Tell the team about allergies, blood thinners, infections and any other cancer medicines. Follow the instructions about eating, drinking and transport; do not stop medicines yourself. Port placement uses local anaesthetic, sometimes with sedation or general anaesthetic. The team explains the procedure and risks, including infection, bleeding, clots and insertion complications. Ask how to manage discomfort, protect the wound and resume activity. Report increasing pain, redness, swelling, discharge or fever.

### Connection: what the nurse does

The nurse checks the line and treatment, secures the connection and dressing, and starts the pump. Before leaving, you should have the disconnection location and time, contact numbers including out of hours, and the written spill instructions. Ask to be shown what normal pump progress looks like. You do not need to calculate a rate.

### Carrying it, sleeping and washing

Use the supplied carrier and the position recommended for your pump. Keep tubing free from pulling and kinks. Ask how to place the pump safely for sleep and how to protect the line and dressing when washing. Do not immerse it or swim. Avoid direct heat and cold. An elastomeric pump has a balloon that shrinks slowly and may have no alarm; an electronic pump has different instructions. Follow the instructions for the device you actually receive.

### Disconnection is not port removal

Keep the appointment even if the pump looks empty, unless the team changes the plan. Staff check the pump and line and arrange safe disposal. After flushing and other line care as required, the port needle may be removed and a small dressing applied. The implanted port stays in place until a separate removal procedure is arranged. A PICC remains in place and needs scheduled dressing and flushing care.

### Leakage, disconnection or unexpected pump progress

Call the team immediately if the pump leaks, is damaged, finishes unexpectedly early, appears not to run, or the line disconnects. Follow the emergency or spill steps you have been specifically taught, including use of a clamp if instructed. Do not reconnect, change settings or disconnect the pump yourself. Keep other people away from leaked medicine. If medicine touches skin, rinse it off and wash with soap and water, then seek advice.

### Pain, swelling or feeling unwell

Contact oncology immediately for pain or swelling around the line, a wet/loose dressing, redness, fever, shivering or suddenly feeling unwell. A painful swollen arm or neck needs urgent advice. Severe chest pain, major breathing difficulty or collapse: call 112 / 999. Do not wait for pump disconnection or the next clinic visit.

The drawings show general steps, not instructions for inserting or accessing a line. Your hospital confirms its own arrangements and the care instructions for your device.

**Open the complete port and pump guide** · **Printable port and pump guide**

Sources: eviQ: port · eviQ: PICC · eviQ: pump. Reviewed against these sources 16 October 2026; local nursing and patient review pending.

PRACTICAL TREATMENT GUIDE

# Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

### Before the first treatment

Confirm the aim, likely course length, blood-test arrangements and a working urgent contact number. DPD testing is required before starting the fluoropyrimidine unless already completed.



STEP 2

### Line and pump arrangements

Confirm venous access, the 46-hour fluorouracil pump and disconnection arrangements before leaving. Know whom to contact for leakage, an alarm, line pain or a pump that finishes unexpectedly.



STEP 3

### If treatment is delayed

Ask the unit to confirm the new treatment and blood-test dates. Re-anchor supportive medicines to the treatment actually given; do not take an extra or catch-up anticancer dose yourself.



STEP 4

### Getting help between visits

Follow your oncology unit’s urgent advice for fever, infection, severe diarrhoea, chest pain or difficulty breathing. Bring your treatment card and current medicines list.

Open practical treatment guide · Printable practical guide

Sources: NCCP 00209 v10a · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

# Support around the medicines

### 1 Nausea prevention and rescue

Take the prescribed prevention medicines at the planned times and keep rescue treatment available. Tell the team if nausea or constipation is not controlled; do not duplicate steroid or anti-sickness medicines.

### 2 Bowel symptoms

Report diarrhoea promptly and follow the written rescue plan. Severe symptoms, fever, dehydration or inability to drink need urgent advice.

### 3 Counts and recovery

White-cell growth-factor injections are prescribed according to individual risk and previous complications. They are not automatically required for every patient or every cycle.

### 4 Cold and nerve symptoms

Avoid cold exposure as advised after oxaliplatin. Report persistent numbness or difficulty with buttons, walking or everyday tasks before the next dose.

### 5 No ice chips with oxaliplatin

Do not suck ice chips during bolus fluorouracil in this regimen: oxaliplatin can cause cold-triggered nerve symptoms. New breathing difficulty still needs clinical assessment.

Sources: nccp

TREATMENT REFERENCE · 01

# Your indication and treatment aim

The same medicines can have different aims.

### After colon-cancer surgery

Adjuvant treatment aims to reduce recurrence risk after complete resection. The likely absolute benefit depends on stage, pathology, fitness and other features. Ask how recurrence risk and neuropathy risk influence the proposed duration.

Sources: nccp

### Advanced colorectal or upper-GI cancer

Treatment may aim to control growth, relieve symptoms and prolong life. The plan is reviewed against response, toxicity and your priorities. A treatment break or a change of components requires an individual discussion.

Sources: nccp

### How long will treatment last?

Do not infer duration from the number of passport pages. Ask for the planned number of cycles and review points. The NCCP schedule distinguishes adjuvant colon treatment, metastatic colorectal treatment and upper-GI treatment.

Sources: nccp

TREATMENT REFERENCE · 02

# Living with the 46-hour pump

Ask for a demonstration and written contact instructions.

### Before leaving

Know what normal pump progress looks like, how to carry it and how to protect the line while washing or sleeping. Check who will disconnect it and when. Have the team check anything you do not understand.

Sources: patient

### Leakage or a delivery problem

Use the hospital spill instructions, avoid direct contact with leaked medicine and call the team. Do not repair a connection or adjust the pump. A pump finishing much sooner or later than expected also needs advice.

Sources: patient

### Chest pain while on fluorouracil

Fluorouracil can affect the heart. New chest pain during treatment needs urgent assessment; severe pain, collapse or major breathing difficulty means 112/999. Tell emergency staff that fluorouracil is infusing. Follow the team's instructions about the pump.

Sources: patient

TREATMENT REFERENCE · 03

## What the checks are for

Symptoms, blood results and the treatment aim answer different questions.

### Blood counts

Neutrophils protect against infection, haemoglobin carries oxygen and platelets help clotting. A normal previous result or feeling well does not prove the current counts are safe. Fever or sudden illness needs immediate advice.

Sources: hse

### Kidney and liver function

Organ function can change drug exposure. Vomiting or diarrhoea can cause dehydration and worsen kidney function. Report reduced urine, poor intake, jaundice and new medicines. The clinical assessment combines results with symptoms.

Sources: nccp

### Describe symptoms in practical terms

Tell the team about the worst symptoms since the last visit, even if improved today. Examples include extra stools above your usual pattern, whether mouth pain affects drinking, and whether numbness affects buttons or balance.

Sources: patient

### How benefit is assessed

After surgery, treatment aims to reduce the chance of recurrence; there may be no visible cancer to measure. In advanced disease, scans, symptoms and function help assess control. Ask when review is planned and what would lead to a change.

Sources: nccp

# Recognising treatment effects

Early reporting helps the team manage symptoms and protect function.

### Acute cold sensitivity and persistent neuropathy

Cold-related tingling can occur soon after oxaliplatin. Persistent numbness between cycles is different and may worsen with repeated treatment. Describe duration and effect on hand use or walking. Some neuropathy can remain after treatment ends.

Sources: patient

### Bowel and mouth symptoms

Fluorouracil can affect rapidly renewing tissues. Keep track of stools compared with your normal pattern, including stoma output. Report diarrhoea, painful ulcers or reduced drinking; do not wait for the next cycle if you are dehydrated or unwell.

Sources: patient

### Hand-foot soreness and skin changes

Tell the team about red, tender or peeling palms or soles. Reduce friction and use the skin care recommended locally. Pain limiting normal activity needs review. Do not assume it must be tolerated to complete treatment.

Sources: patient

### Fatigue and nausea

Use manageable activity, rest and the prescribed anti-sickness plan. New profound weakness, faintness or breathlessness warrants assessment because anaemia, infection, dehydration and other problems can look like ordinary fatigue.

Sources: hse

# Living with treatment

Plan around the activities and people that matter to you.

### Food, drink and practical help

Use foods and drinks you can manage. Tell the team about weight loss, poor intake or difficulty shopping and cooking; ask for dietitian or practical support. A fixed daily fluid target is not appropriate for everyone, particularly with heart or kidney problems.

Sources: patient

### Activity, rest and driving

Balance rest with manageable activity. Record what fatigue stops you doing. Do not drive when dizzy, unwell or affected by sedating medicines. Ask about work adjustments and transport for treatment days.

Sources: patient

### Medicines, vaccines and supplements

Bring a complete medicine list, including over-the-counter products and supplements. Ask the oncology pharmacist before changes. Discuss vaccination timing. Do not stop prescribed long-term medicines on your own.

Sources: hse

### Fertility, contraception and breastfeeding

Discuss fertility preservation before treatment when relevant. Ask for a written contraception and breastfeeding plan covering each medicine and the period after treatment. Tell the team promptly about a possible pregnancy.

Sources: hse

### Support for you and your family

Ask about psycho-oncology, social work, counselling and symptom support. Take information in manageable amounts. A family member or trusted person can help record questions if you wish. Discuss how treatment fits your priorities.

Sources: patient

TREATMENT REFERENCE · 06

## Words you may hear

A short reference for clinic discussions.

### Adjuvant, metastatic and cycle

Adjuvant means treatment after the main treatment, usually surgery, to reduce recurrence risk. Metastatic means spread to a distant site. A cycle includes treatment and the interval before the next planned start.

Sources: nccp

### DPD, neuropathy and dose modification

DPD is an enzyme involved in fluorouracil breakdown. Neuropathy means nerve damage or dysfunction. Dose modification means changing the amount, timing or components to balance benefit and harm.

Sources: nccp

## Understanding the evidence

Ask which evidence matches your cancer, treatment aim and medicines.

Adding oxaliplatin to fluorouracil-based treatment improved long-term outcomes in a study of people with stage III colon cancer after surgery. That study used FOLFOX4, a related but different schedule. It does not predict benefit for every indication of modified FOLFOX6 or for an individual person.

Sources: mosaic

PREPARE FOR THE CONVERSATION

## Questions worth asking

### Does having more side effects mean treatment is working?

No. Side effects do not reliably show whether the cancer is responding. Report them so they can be treated. Your team assesses benefit using the tests and review plan appropriate to your cancer.

### Why might a dose be delayed or reduced?

Your team may allow recovery, investigate a symptom or reduce the chance of lasting harm. Ask which drug changes, why, and when review is planned. A delay does not itself mean treatment has failed.

**What should I say when calling?**

Give the regimen name, last treatment date, symptoms and when they started. Explain whether you can eat, drink, pass urine and manage usual activities. Give your temperature if available, but do not delay calling to collect details.

**What belongs in the consent conversation?**

The individual aim and likely benefit, uncertainty, reasonable alternatives including not proceeding, common and serious risks, lasting effects, fertility and practical concerns. You can ask for an interpreter, more time or another explanation and can reconsider your decision.

**How many cycles should I use this passport for?**

The passport provides reusable recording pages, not a prescribed course length. Your oncology team sets the duration and can provide extra pages.

**Can I use this page to decide if blood results are safe?**

No. Treatment decisions require the clinical protocol, symptoms, previous toxicity, organ function and the prescriber's review. The patient page explains the checks and does not authorise treatment.

> **Try explaining the plan back:** Can you explain which medicines you are receiving, the schedule, the contact plan and the symptoms that need prompt help?

SOURCE TRANSPARENCY

# Where this information comes from

Regimen schedule: NCCP 00209 v10a. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00209 v10a · official regimen
- eviQ: patient information (supportive explanation; NCCP schedule takes priority)
- HSE: chemotherapy side effects and urgent advice

- NCI: nausea and vomiting
- MOSAIC: ten-year follow-up (2015)
- eviQ: treatment through an implanted port
- eviQ: treatment through a PICC
- eviQ: elastomeric infusion pump care

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SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-04.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

## Written source addresses

https://healthservice.hse.ie/documents/6549/209_v10a_FOLFOX_6_Modified.pdf

https://www.eviq.org.au/medical-oncology/colorectal/metastatic/114-colorectal-metastatic-folfox6-modified-fluo/patient-information

https://www.eviq.org.au/p/507

https://www.eviq.org.au/p/543

https://www.eviq.org.au/p/502

https://www2.hse.ie/conditions/chemotherapy/side-effects/

https://www.cancer.gov/about-cancer/treatment/side-effects/nausea-vomiting

https://pubmed.ncbi.nlm.nih.gov/26527776/

BASELINE → TREATMENT → FOLLOW-UP

# Checks across the whole treatment course

The columns show the order of care, not equal lengths of time. Confirm who books each check, who reviews it and whom to call if it is missing.

| Check | Before starting | Each treatment cycle | Between doses | Decision / response review | Completion / change |
|---|---|---|---|---|---|
| **Blood count / kidneys / liver** | Baseline bloods; DPD before fluoropyrimidine | Repeat before each cycle | Earlier tests if unwell, dehydrated or toxicity develops | Results and recovery guide any delay or dose change | Follow unresolved abnormalities |
| **Symptoms and function** | Document existing symptoms and usual activity | Review worst symptoms since the last treatment | Use urgent contacts; do not wait for the next visit | Balance benefit, side effects and priorities | Agree recovery care and ongoing support |
| **Neuropathy / cold sensitivity** | Record existing nerve symptoms | Check persistence and effect on daily tasks | Report worsening numbness or cold-triggered throat symptoms | Consultant reviews the oxaliplatin component separately | Nerve symptoms can continue after treatment; arrange review |
| **Heart checks** | Review cardiac history; ECG when cardiac function is compromised or clinical findings warrant it. No automatic serial echocardiogram schedule is specified for this regimen. | Ask about new chest pain or breathlessness | Chest pain needs urgent assessment | Repeat heart tests if clinically indicated | Follow any heart problem; no fixed echo series |
| **Response / scans** | Confirm cancer setting, disease assessment and intended duration | Review clinical benefit and symptoms | New symptoms may bring review forward | Consultant-directed scans and response assessment | Agree follow-up, completion or another treatment |

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00209 v10a · official regimen

THE WHOLE TREATMENT JOURNEY

# Modified FOLFOX-6: treatment and checks

## Gastrointestinal cancers

Day 1: oxaliplatin, folinic acid and fluorouracil, followed by a 46-hour fluorouracil pump. A cycle lasts 14 days.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | Each treatment cycle | Between doses | Decision / response review | Completion / change |
|---|---|---|---|---|---|
| **Blood count / kidneys / liver** NCCP 00209 | Baseline bloods; DPD before fluoropyrimidine | Repeat before each cycle | Earlier tests if unwell, dehydrated or toxicity develops | Results and recovery guide any delay or dose change | Follow unresolved abnormalities |
| **Symptoms and function** NCCP 00209 | Document existing symptoms and usual activity | Review worst symptoms since the last treatment | Use urgent contacts; do not wait for the next visit | Balance benefit, side effects and priorities | Agree recovery care and ongoing support |
| **Neuropathy / cold sensitivity** NCCP 00209 | Record existing nerve symptoms | Check persistence and effect on daily tasks | Report worsening numbness or cold-triggered throat symptoms | Consultant reviews the oxaliplatin component separately | Nerve symptoms can continue after treatment; arrange review |
| **Heart checks** NCCP 00209 | Review cardiac history; ECG when cardiac function is compromised or clinical findings warrant it. No automatic serial echocardiogram schedule is specified for this regimen. | Ask about new chest pain or breathlessness | Chest pain needs urgent assessment | Repeat heart tests if clinically indicated | Follow any heart problem; no fixed echo series |
| **Response / scans** NCCP 00209 | Confirm cancer setting, disease assessment and intended duration | Review clinical benefit and symptoms | New symptoms may bring review forward | Consultant-directed scans and response assessment | Agree follow-up, completion or another treatment |

# What each check is for

1

### Confirm your personal plan

Before starting

Modified FOLFOX-6 combines oxaliplatin, folinic acid and fluorouracil. It can be used after colon-cancer surgery or to treat advanced colorectal, stomach or gastro-oesophageal-junction cancer. The aim and duration depend on the indication; ask which applies to you.

**Ask / act:** Agree medicines, treatment days, duration and monitoring.

Sources: NCCP 00209

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-04 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.32

# Modified FOLFOX-6

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Separate scripts for each phase

Choose the active phase in the clinician workflow. Each script has its own medicines, timing and review checks.

- **Modified FOLFOX-6 · active treatment cycle** — Moderate
  Oxaliplatin + Folinic acid + Fluorouracil

https://sactcheck.com/patient/00209/supportive-care.html

# Modified FOLFOX-6

NCCP 00209 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

## Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

### Before the first treatment

Confirm the aim, likely course length, blood-test arrangements and a working urgent contact number. DPD testing is required before starting the fluoropyrimidine unless already completed.

STEP 2

### Line and pump arrangements

Confirm venous access, the 46-hour fluorouracil pump and disconnection arrangements before leaving. Know whom to contact for leakage, an alarm, line pain or a pump that finishes unexpectedly.

STEP 3

### If treatment is delayed

Ask the unit to confirm the new treatment and blood-test dates. Re-anchor supportive medicines to the treatment actually given; do not take an extra or catch-up anticancer dose yourself.

STEP 4

### Getting help between visits

Follow your oncology unit’s urgent advice for fever, infection, severe diarrhoea, chest pain or difficulty breathing. Bring your treatment card and current medicines list.

Sources: NCCP 00209 v10a · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

SACTCHECK · TREATMENT PASSPORT

# Modified FOLFOX-6

## A record of what you experience

One cycle per page. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal CTCAE grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Day 1: oxaliplatin, folinic acid and fluorouracil, followed by a 46-hour fluorouracil pump. A cycle lasts 14 days.

The four pages are reusable recording sheets, not a prescribed four-cycle course. Ask for additional sheets as needed.

**Emergency: 112 / 999.** Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

**Contact oncology immediately** for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

**Insert numbers here**

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

**Insert numbers here**

Ask for medical oncology

EVENINGS / OVERNIGHT

**Your out-of-hours pathway / nearest ED**

Other hospitals: use the numbers your team provides.

Your own hospital / contact instructions:

https://sactcheck.com/patient/00209/

Full patient information, sources and visual timeline. No identifiers are encoded.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

# Modified FOLFOX-6

Cycle number: _____ Start date: _____ Next review: _____

### Oxaliplatin · Platinum chemotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Numbness / tingling** | ☐ None | ☐ Mild change; usual tasks unaffected | ☐ Limits buttons, writing or walking: call | ☐ Washing, dressing or walking difficult: urgent review | ☐ Sudden major weakness / paralysis: emergency help |
| **Nausea** | ☐ None | ☐ Queasy but eating and drinking | ☐ Intake reduced: contact the team | ☐ Unable to maintain intake: urgent review | ☐ Collapse / severe dehydration: 112/999 |
| **Fatigue** | ☐ Usual energy | ☐ Tiredness improves with rest | ☐ Rest does not help; usual tasks affected: call | ☐ Self-care difficult: urgent review | ☐ Collapse: 112/999 |
| **Diarrhoea** | ☐ Usual pattern | ☐ Up to 3 extra stools above normal | ☐ 4–6 extra stools/day or at night: call | ☐ 7+ extra stools/day or dehydration: urgent review | ☐ Collapse / severe dehydration: 112/999 |

### Folinic acid · Chemotherapy modulator

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Diarrhoea** | ☐ Usual pattern | ☐ Up to 3 extra stools above normal | ☐ 4–6 extra stools/day or at night: call | ☐ 7+ extra stools/day or dehydration: urgent review | ☐ Collapse / severe dehydration: 112/999 |
| **Mouth soreness / ulcers** | ☐ None | ☐ Mild soreness; eating normally | ☐ Painful; soft food needed: call | ☐ Cannot eat / drink enough: urgent review | ☐ Major breathing difficulty: 112/999 |

### Fluorouracil · Chemotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Diarrhoea** | ☐ Usual pattern | ☐ Up to 3 extra stools above normal | ☐ 4–6 extra stools/day or at night: call | ☐ 7+ extra stools/day or dehydration: urgent review | ☐ Collapse / severe dehydration: 112/999 |
| **Mouth soreness / ulcers** | ☐ None | ☐ Mild soreness; eating normally | ☐ Painful; soft food needed: call | ☐ Cannot eat / drink enough: urgent review | ☐ Major breathing difficulty: 112/999 |
| **Fatigue** | ☐ Usual energy | ☐ Tiredness improves with rest | ☐ Rest does not help; usual tasks affected: call | ☐ Self-care difficult: urgent review | ☐ Collapse: 112/999 |
| **Hand-foot soreness** | ☐ None | ☐ Redness or tingling without pain | ☐ Pain limits usual tasks: call | ☐ Pain limits washing / dressing: urgent review | ☐ Acute severe illness: emergency help |

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

# Modified FOLFOX-6

Cycle number: _____ Start date: _____ Next review: _____

### Oxaliplatin · Platinum chemotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Numbness / tingling** | ☐ None | ☐ Mild change; usual tasks unaffected | ☐ Limits buttons, writing or walking: call | ☐ Washing, dressing or walking difficult: urgent review | ☐ Sudden major weakness / paralysis: emergency help |
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### Folinic acid · Chemotherapy modulator

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|---|---|---|---|---|---|
| **Diarrhoea** | ☐ Usual pattern | ☐ Up to 3 extra stools above normal | ☐ 4–6 extra stools/day or at night: call | ☐ 7+ extra stools/day or dehydration: urgent review | ☐ Collapse / severe dehydration: 112/999 |
| **Mouth soreness / ulcers** | ☐ None | ☐ Mild soreness; eating normally | ☐ Painful; soft food needed: call | ☐ Cannot eat / drink enough: urgent review | ☐ Major breathing difficulty: 112/999 |

### Fluorouracil · Chemotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Diarrhoea** | ☐ Usual pattern | ☐ Up to 3 extra stools above normal | ☐ 4–6 extra stools/day or at night: call | ☐ 7+ extra stools/day or dehydration: urgent review | ☐ Collapse / severe dehydration: 112/999 |
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Cycle number: _____ Start date: _____ Next review: _____

### Oxaliplatin · Platinum chemotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
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### Folinic acid · Chemotherapy modulator

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Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

# Keep your treatment details to hand

**Gastrointestinal cancers** · NCCP 00209 v10a · Document revised 4 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

**FRONT / CONTACTS**

## My Treatment Card

SACTCheck

Name: _____

**Modified FOLFOX-6**

Day 1: oxaliplatin, folinic acid and fluorouracil, followed by a 46-hour fluorouracil pump. A cycle lasts 14 days.

Phase: _____ Cycle plan: team to confirm

**Daytime (EXAMPLE)** 087 12345678

**Out of hours (EXAMPLE)** 087 87654321

**Unwell or fever? Call oncology now.**
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

**BACK / TREATMENT RECORD**

## My cycle record

Rolling record — follow your agreed treatment plan.

| Cycle | Date / initials |
|---|---|
| | |
| | |
| | |
| | |

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

CUT outer border

**FOLD centre — do not cut**

## Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

## Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

### iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

### Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

## Your local contacts and resources

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**

Hospital / team: _____
Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

SACTCheck v0.84.32 · NCCP 00209 v10a · Wallet-card sheet
Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.